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Jewish circumcision: an alternative perspective

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Introduction

As a Jew, I used to take ritual circumcision completely for granted. I watched the procedure many times, when a son was born to family or friends. It is unusual for a woman to watch, but I was a young medical student, unafraid, curious and, eventually, horrified. My concern about this issue has arisen slowly, simply from what I have seen.

In the first section, I examine the reasons for questioning ritual circumcision, and compare some of the physiological and clinical findings about the procedure with popular Jewish beliefs about its nature and consequences. I consider the effect of the procedure on the mother as well as the child, and discuss the ethical implications. In the second section, I discuss some Jewish texts and traditions that could, surprisingly, lend support to challenging the established practice of circumcision.

In the third section, I describe the historical and psychosocial factors that make challenging Jewish circumcision so problematic, and that sometimes lead to intensely negative responses to new information about it. I also discuss the vexed question of whether challenging Jewish circumcision is necessarily anti-Semitic.

In the penultimate section, I examine the received view that Jewish people practise circumcision because of God's commandment to Abraham (in Genesis 17: 11–14). I show that for most Jewish people, the reasons have more to do with a sense of belonging than with a sense of being divinely commanded. I then argue that circumcision is, nevertheless, not as essential to our Jewish identity as most of us have assumed. Finally, I consider the possibility of cultural evolution, providing examples from within and outside the Jewish community of how, despite the difficulties, customs can and do change.

Reasons for questioning religious circumcision

There are several reasons to reconsider the practice of religious circumcision, including the pain, trauma and complications associated with the procedure, the lack of previously supposed medical benefits, and the effect of circumcision upon the mother of the circumcised child.

Pain and trauma

Jewish boys are circumcised at 8 days of age, i.e. well within the neonatal period; no anaesthetic is used. The level of pain associated with the operation, whether judged by the babies' screams or by objective physiological indices such as heart rate and plasma cortisol levels, is severe. Even using a lignocaine injection for neonatal circumcision, Stang *et al.* [1] showed plasma cortisol levels of 331 nmol/L, whereas the normal level is 28–138 nmol/L. Similarly, Benni *et al.* [2] found heart rates of 180 bpm in recently circumcised babies, which were only slightly reduced (to 160) by anaesthetic. Paediatrician Paul Fleiss, writing in the *Lancet*, considers these cortisol levels and heart rates to be 'consistent with torture' [3].

Other researchers have found comparable physiological responses indicative of severe stress during circumcision, including dramatic rises in respiratory rate and transcutaneous oxygen, as well as plasma cortisol level and heart rate [4–6]. Circumcision is so painful that it has now, according to Stang *et al.* 'become a model for the analysis of pain and stress responses in the newborn' [1].

These findings contrast starkly with the beliefs commonly expressed within the Jewish community that 'it's only a little snip that doesn't hurt', and 'it's just like changing their nappy or cutting their fingernails'. Even though most babies scream, tremble and go pale or even blue, many Jewish people will describe a circumcision they attended at which 'the baby didn't scream at all, he didn't even cry, in fact he just went to sleep'. This is taken to mean that the baby felt no pain. However, studies suggest that rather than feeling fine, these babies may actually be lapsing into a lethargic, semicomatose state as a reaction to overwhelming stress [7–9].

Another popular belief, perpetuated even by those *Mohelim* (ritual circumcisers) who are also doctors, is that 'there are no nerve endings in the foreskin, so it can't possibly hurt'. However, it has been clearly shown that the foreskin is a complex and important sensory organ, containing many nerve endings, including the specialized Meissner's corpuscles, similar to those found in the fingertips and lips [10].

In conversations about circumcision, many Jewish people claim that Jewish circumcision is less traumatic than, say, Muslim circumcision, precisely because it is performed at such an early age. The underlying myth, common until quite recently in the wider community, is that newborn babies simply do not feel pain. But medical research contradicts the wishful thinking of folk beliefs. Not only do newborn babies feel pain, they feel it more

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intensely, for longer and over a wider area of the body than do older children or adults subjected to the same stimulus. Fitzgerald and her team at University College London, have shown clearly that the nervous system of a newborn baby differs from that of an older child or adult, both anatomically and physiologically, so that what would constitute a light or harmless stimulus to the older child or adult actually produces pain in the newborn. Furthermore, newborns lack the inhibitory or 'damping down' mechanisms of the more mature nervous system, so they cannot protect themselves from the experience of pain in the way they could at a later stage [11].

The developmental anatomy of the penis also suggests that the earlier circumcision is performed, the more painful it will be. In the neonatal period, only 4% of children have a retractable foreskin. For 96% of Jewish babies therefore, circumcision will involve the forcible tearing of the foreskin from its attachment to the glans, as well as the cutting of its attachment to the sulcus. However, at the age of 3 years, 90% of boys have a retractable foreskin, and therefore this additional trauma would only apply to 10% of them [12].

Closely following (and contradicting) the 'no pain' story, one frequently encounters the following statement: 'Alright, maybe it hurts, but it only lasts a moment and then they forget all about it the minute it's over. They're too young to have any memory of it'. In fact, the raw surface of the glans may bleed and be painful for several days after circumcision, so the pain is far from momentary. Further, there is ample evidence that newborns do have some memory of the event, which takes the form, not of conscious remembering, but of a permanent restructuring of the nervous system. The result is an intensification of the behavioural response to subsequent painful stimuli, as though the nervous system has been 'sensitized'. Thus, for example, Taddio *et al.* [13] found that pain responses in children being vaccinated were significantly greater in those who had been circumcised (several months previously) than in those who had not. Other studies show that pain experienced in early infancy can disrupt breast-feeding, mother-infant bonding [14] and sleeping patterns [15].

In addition to pain, circumcision is traumatic because it constitutes an experience of utter helplessness [16]. Not only is the child in great pain, but nothing he does can help to avoid or reduce the pain. His cries have no effect and movement to escape is impossible, because he is held down; he learns that he is powerless.

Complications

Many serious complications of ritual circumcision have been reported [17], including haemorrhage, infection, meatal stenosis [18], amputation of the penis [19,20], trapped penis [21], urinary retention (frequently, from tight bandaging) and renal impairment (rarely) [22], UTI [23], septicaemia [24], and four cases of acute heart failure [25]. Williams and Kapila [26] reported a horrifying range of surgical disasters leading to permanent severe damage to the penis.

I have listened to several Jewish women describe how their sons haemorrhaged severely after circumcision, requiring hospital admission and blood transfusion. In one case, an orthodox Jewish woman, whose only child had bled nearly to death, felt so traumatized that she chose to have no more children, although deeply wanting them. She said 'if I had another boy, I couldn't bear to have him circumcised. But I also couldn't bear *not* to have him circumcised'. Her son is 16 years old now and has no siblings.

While haemorrhage, infection and the occasional tragic accident may be acceptable risks in surgery performed to treat disease, and to save life and limb, i.e. where the benefits clearly outweigh the risks, they are unacceptable risks in 'surgery' that has no such healing function. As far as I have been able to ascertain, neither the Jewish nor the Muslim community in Britain keep any statistical records of the complications of circumcisions performed by their practitioners.

Lack of presumed medical benefits

According to the Jewish religion, circumcision should be carried out purely for reasons of faith, not for any real or imagined medical benefits. Nevertheless, in conversation with secular and non-orthodox Jewish people, 'hygiene' and 'preventive medicine' are frequently given as reasons for continuing the practice. Most Jewish people seem unaware that claims for the prevention of cervical cancer, penile cancer, UTI, etc., have been thoroughly discredited [27], as were the claims for prevention of asthma, epilepsy and 'masturbatory insanity' which preceded them. In any case, preventive medicine is normally about choices that a free adult can make, relating to diet, lifestyle, screening programmes, etc. It is not usually about removing healthy body parts from infants.

Effects on the mother

The strongest instinct of the new mother is to hold, cherish and protect her baby. In the Jewish world, this natural and vital desire is brought into immediate and painful conflict with the demands of tradition. In gatherings of Jewish women over the last several years, I have listened to scores of Jewish mothers and grandmothers describing how they felt torn between loyalty to their

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child and loyalty to their tradition, at what should have been a time of resting and rejoicing. I have heard African and Muslim mothers describe the same agonizing dilemma. Usually, the circumcision goes ahead and the mother feels impotent, guilty and full of regret, even many decades later. Many mothers say they only consented because they had been told it was a painless, minor procedure. Had they realized the degree of blood and trauma involved, they would never have agreed; they felt betrayed.

In Jewish law it is the father, not the mother, who is commanded to circumcise the boy. It is a rite of male bonding, designed (unconsciously), as are all such rites worldwide, to separate the baby boy from his mother and to claim him as 'one of the men' of the tribe. It was Abraham, not Sarah, who was commanded to circumcise. It is Jewish men, not the women or children, who have defined circumcision as sacred. As Pollack points out [28], Jewish women now need to redefine what is sacred, and every mother knows in her heart that there is nothing sacred about taking a knife to a child's genitals. There is nothing sacred about shattering a child's inborn trust, his basic assumption that his parents will do their utmost to protect him from harm. Pollack states, 'Circumcision is fundamental to patriarchy, but it is not holy' [29].

Ethical implications

Anyone subjecting an adult to what is permitted in regard to an 8-day-old baby would be charged with assault. If children and babies are to be regarded as fully human beings, then it must surely be unethical for them to be held down forcibly while a healthy, functional part of their body is excised. It is not only that babies are helpless, or that they are too young to give consent. Their reactions, their screams, and that they have to be held down at all, are their best and only way of indicating that they are actually *withholding* their consent.

Judaism, like all religious traditions, has many layers and many strands. In the next section, I show that despite the Biblical injunction to circumcise, support can be found from many Jewish sources for the view that circumcision of infants is unethical, and should therefore be abandoned.

Textual support for change from within Judaism

In Judaism, and in Islam, the human being is considered to be made in the image of God, and God is conceptualized as perfect. So one could argue that interfering with God's perfect creation is a form of blasphemy. In Judaism there is a law of '*Shmirat Ha Guf*', the guarding or protecting of the body. Body-piercing, tattooing and amputation are all forbidden for this reason. Further, there is the Talmudic concept of '*Tsa'ar ba'alei chayyim*', compassion for all living creatures. If compassion in all its fullness were applied to 8-day-old babies, circumcision would become impossible.

The literal commandment to circumcise that is found in Genesis reappears in the later books of Deuteronomy and Isaiah with a metaphorical meaning. The prophet Isaiah urges people to 'circumcise the foreskin of your hearts', i.e. to melt the hard shell of defensive insensitivity with which each of us armours our innermost selves. This is highly applicable today; as we 'circumcise the foreskins of our hearts', we become able to hear the anguished cries of our babies.

The *Shulchan Aruch*, the authoritative Code of Jewish Law, says: 'Extreme care should be taken not to circumcise an infant who is ailing, as *the fulfilment of all ordinances is suspended if there is danger to human life*. Moreover, the circumcision can be performed at a later date, but the life of a human being can never be restored'. [30] (emphasis added by the present author).

The Talmud discusses at length the imperative of '*Pikuach Nefesh*', the obligation to save human life in any situation in which it is endangered, whether the danger is immediate or only potential, as in the case of a condition which is not dangerous now but could suddenly deteriorate [31]. The rabbis derive this ruling from Leviticus 19:16; 'Neither shalt thou stand idly by the blood of thy neighbour'. It could certainly be argued that circumcision of an 8-day-old baby constitutes a condition which could, and sometimes does, suddenly deteriorate.

The Talmud goes on to say (Hul. 10a) that 'One should be more particular about matters concerning life and health than about ritual observances'. It insists, for example, that even the laws of the Sabbath *must* be broken to give medical treatment or comfort to a sick person or a postpartum woman (Yoma 85a). This is despite the fact that keeping the Sabbath is one of the Ten Commandments; circumcision, significantly, is not.

Jewish law is an evolving process that has always taken into account new developments in science and understanding, and attempted to integrate them. Given what is known about the life-threatening complications of neonatal circumcision, there is an argument from within Judaism to adapt Jewish law, so that the circumcision of helpless, non-consenting babies becomes forbidden, not demanded.

The medieval rabbi Maimonides, regarded as the greatest Jewish sage of all time, writes of circumcision: ' ... ... one of its objects is to limit sexual intercourse, and to weaken the organ ... the use of the foreskin to that organ is evident ... there is no doubt that circumcision weakens the power of sexual excitement, and sometimes lessens the natural enjoyment; the organ necessarily becomes weak when it loses blood and is deprived of its

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covering from the beginning' [32]. Maimonides was a physician as well as a rabbi. His words foreshadow current medical understanding of the protective and sexual functions of the foreskin. Although he was writing in defence of circumcision, the text might be interpreted very differently today.

Difficulties of questioning circumcision in the Jewish community

Many Jewish people, when they encounter the arguments outlined in the first section about the pain, trauma and dangers of circumcision, say something along the lines of: 'Well, you're quite right. I can't disagree with you. But — I know it's completely irrational, but I just feel we must carry on doing it'. Others become very angry, dismissing solid medical evidence and genuine humane concern as an anti-Semitic attack. To make sense of these responses, and of the tremendous resistance (i.e. fear manifesting as defensiveness) which will be encountered by any person challenging Jewish circumcision, whether from within or outside the community, the psychological meaning of circumcision for Jewish people must be understood. To understand that requires some knowledge of Jewish history.

Persecutors of the Jews have always attempted to ban circumcision. That their motivation was the extermination of the Jewish people, rather than any humanitarian concern for children, is shown by the fact that circumcision was frequently punished by death. In 168 BCE, Antiochus IV Epiphanes of Syria ordered his soldiers to execute circumcised Jewish babies and their parents. In CE 135, the Roman Emperor Hadrian forbade Jewish religious teaching and the keeping of the Sabbath, and made circumcision an offence punishable by death. Jews defied these decrees, and thousands were tortured and died the death of martyrs, refusing to abandon this symbol of their faith. The story was repeated in the Spanish Inquisition, under the Soviet Government and under the Nazis. Circumcision thus became an emblem not just of faith, but of loyalty, courage and the defiant fight for freedom against terrible oppression. Even in the cattle-cars on the way to Auschwitz, Jews circumcised their children. They were doing what they believed to be right, affirming their identity and faith even in the depths of hell.

All Jewish people, including myself, carry this history in our collective memory, and any challenge to circumcision evokes it, whether consciously or unconsciously. Even the most well-intentioned criticism can therefore sound like a massive threat. Circumcision has become, in Pollack's words, 'laminated to our psyches by generations upon generations of trauma'. [28].

As well as those features that are specific to Jewish history, there are other powerful reasons why ritual circumcision continues, i.e. psychological mechanisms common to any communities practising circumcision or equivalent rites. Familiarity breeds numbness and denial. The men present at a *Bris Milah* (Covenant of Circumcision; women are usually absent, from choice in the Reform Jewish world and by exclusion in Orthodox circles) have all been circumcised themselves. To them, it is the 'normal' state. They are present at a 'celebration', bringing their child into the Covenant. In a very real sense, they do not hear the child's screams of agony. This desensitization is a cross-cultural phenomenon; it was apparent in the obstetric wards of the 1970s and 1980s, where nurseries full of screaming newborns, isolated from human touch in cribs far away from their mothers, were approvingly dismissed with phrases such as 'healthy pair of lungs'. This was also 'normal'. For the circumcised Jewish man, therefore, it takes tremendous courage and insight to say 'I have been damaged. My parents made a mistake. I won't pass this wound on to my son'.

Fear of anti-Semitism must not be allowed to stifle dissent. Nevertheless, for those choosing to challenge Jewish circumcision from outside, it is necessary to know the history outlined above and to be aware of how any criticism may therefore be misinterpreted. It is also crucial to be absolutely certain that the message and motivation are free from racism in the present. Specifically, criticising a custom must not degenerate into attacking or demonizing those who practise it. They are good-hearted people who love their children, but have grown up with a different set of rules. Most people of all races and religions do what they have always done, unquestioningly, most of the time, and Jews, Muslims and others who circumcise are no exception. One way to avoid racism is to take the broad, multicultural view, that we all have something to learn from each other. An example will make this clearer; many African mothers have their daughters genitally mutilated. White westerners find this appalling and wonder secretly whether the parents can really love their daughters. But then, many white western parents even now leave their young children to sob, alone, behind a closed door, night after night, because it is past 7.30 pm and 'they must learn that bed-time means bed-time'. Any African mother would be appalled and would wonder secretly whether the parents can really love their children. She would tell us that young children need human contact, carrying and cuddling, whatever time of day or night; she would tell us that we too are doing something wrong.

Anti-Semitism is a particularly virulent form of racism and avoiding it while challenging Jewish circumcision must be a matter of deep intent, not merely of style. This is not only because racism is counterproductive in the

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effort to educate people fully about the reality of circumcision, but because racism is itself as cruel and unethical as the circumcision of helpless babies. I am challenging circumcision because it is the particular form of violence to children that occurs in my own culture; however, every culture has its own forms of oppression that needs attention.

Religious circumcision and Jewish identity

The usual defence advanced for Jewish circumcision is that it is commanded by God (Genesis 17: 11–14), so it simply must be done. However, on closer examination, this statement (which is in any case not amenable to rational argument) turns out not to be the real reason, except for the tiny minority of Jews who take the Bible literally. Surveys show that even among traditional Jewish women (those attending orthodox synagogues), only 32% consider belief in God to be central to being a good Jew [33], and the value would be even lower if it included men, as women are more likely than men to believe in God [33]. Most Jewish people do not carry out most of the 613 biblical commandments. Even the ultra-orthodox would not dream of carrying out the whole of the circumcision commandment, which actually states that servants and non-Jewish house-guests must also be circumcised! (Genesis 17:12).

The inconsistency is even more striking when considering Reform, Liberal, atheist and unaffiliated Jewish people. The Reform and Liberal movements select freely among commandments, modernizing the tradition and re-interpreting biblical texts in metaphorical, socially conscious ways. Atheist and unaffiliated Jewish people have often abandoned all contact with the tradition. Yet all these groups still circumcise their sons, indicating that they feel psychologically just as 'bound' as the strictest traditionalist. What binds them is not a sense of divine command, but a fear of not belonging; there is a real terror of being the one to 'break the chain'. The weight of history and the resultant cultural pressure mediated through the family are very difficult forces to resist.

For all these reasons, and those discussed in the previous section, circumcision seems to be inextricably welded to Jewish identity, and challenges to that identity are experienced as challenges to Jewish survival itself. Yet there are arguments to show that Jewish identity is not, after all, as dependent upon continuing circumcision as it might appear.

First, whose Jewish identity? Only that of the men. Jewish women have maintained their identity perfectly well for 4000 years, with no equivalent marking or scarring, and to say that 'circumcision is vital to Jewish identity' is to exclude 52% of the Jewish population. Second, although Jewish people like to think of circumcision as an identifying sign unique to them, it clearly is not. It is practised also by Muslims, non-Muslim Africans, Australian Aborigines and much of white North America (and Korea, see Kim and Pang, this supplement), and is described in Egyptian papyri from 4000 BCE. Thirdly, although some rabbis still believe that circumcision will keep Jewish males 'within the fold', the reality is that it has not stopped many young Jewish men from leaving the faith. Lastly, in Jewish law (*Halacha*), the son of a Jewish mother is *de facto* Jewish, whether circumcised or not.

Conclusion: Possibilities of cultural change

All cultures evolve and change, and what was indispensable in one era becomes redundant in the next. Generations of Chinese women were crippled by foot-binding, a practice that was considered absolutely integral to the Chinese way of life, until the 1920s, when feminist campaigners successfully stopped it, and the predicted 'collapse of the culture' failed to materialize. Similarly, the Sabine people of Eastern Uganda have recently abandoned their long-entrenched, traditional practice of female genital mutilation; their culture, too, has survived.

In the British Jewish community, a small but growing number of young parents are refusing to circumcise their baby boys. They are beginning to create alternative, non-violent rituals, gentle ceremonies to welcome babies of both sexes into the Jewish community and the wider world. The greatest fear of Jewish parents, that their son will be ostracized from the community if he is not circumcised, turns out to be a circular, self-perpetuating argument, only true so long as everyone believes it. As more parents have the courage to say 'no', then the intact Jewish boy becomes no longer the exception, no longer the 'odd one out'. In fact, the uncircumcised Jewish boys and their families have remained firmly within the Jewish community. Judaism has many joyful and life-affirming customs to maintain and pass on, and I believe Jewish culture will be strengthened, not weakened, by our outgrowing of neonatal circumcision.

I have shown that circumcision is painful, traumatic and potentially dangerous, but that because of our past, it carries huge symbolic significance in the collective Jewish psyche. However, I suggest that our tremendous historical suffering does not negate or justify the continuing pain of our baby boys. Chief Rabbi Dr Jonathan Sacks, interviewed on the BBC [34], stated that the purpose of circumcision was to 'sanctify sexuality'. When asked how it could do so, he replied, 'It's not causal, it's symbolic'. However, in the final analysis, circumcision is not symbolic for the baby; it is horribly real. Now is

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the time to lay the knife aside and to move forward into the 21st century with a form of ritual that is truly welcoming and that is truly purely symbolic.

References

1 Stang HJ, Gunnar MR, Snellman L, Condon LM, Kestenbaum R. Local anaesthesia for neonatal circumcision: Effects on distress and cortisol response. *JAMA* 1988; **259**: 1507–11
2 Benni F, Johnston C, Faucher D, Aranda JV. Topical anaesthesia during circumcision in new-born infants. *JAMA* 1993; **270**: 850–3
3 Fleis PM. Letter. *Lancet* 1995; **345**: 927
4 Rawlings DJ, Miller PA, Engle RR. The effect of circumcision on transcutaneous PO_2 in term infants. *Am J Dis Child* 1980; **134**: 676–8
5 Gunnar MR, Fisch RO, Korsvik S *et al.* The effects of circumcision on serum cortisol and behaviour. *Psychoneuro-Endocrinology* 1981; **6**: 269–75
6 Talbert LM, Kraybill EN, Potter HD. Adrenal cortical response to circumcision in the neonate. *Obstet Gynecol* 1976; **48**: 208–10
7 Emde RN, Harmon RJ, Metcalf D *et al.* Stress and neonatal sleep. *Psychosom Med* 1971; **33**: 491–7
8 Gunnar MR, Malone S, Vance G *et al.* Quiet sleep and levels of plasma cortisol during recovery from circumcision in newborns. *Child Dev* 1985; **56**: 824–34
9 Marshall RE, Stratton WC, Moore JA *et al.* Circumcision: 1. Effects on newborn behaviour. *Infant Behav Dev* 1980; **3**: 1–14
10 Taylor JR, Lockwood AP, Taylor AJ. The prepuce: Specialised mucosa of the penis and its loss to circumcision. *Br J Urol* 1996; **77**: 291–5
11 Fitzgerald M. The birth of pain. *MRC News* 1998; **78**: 20–3
12 Gairdner D. The fate of the foreskin. *Br Med J* 1949; **2**: 1433–7
13 Taddio A, Goldbach M, Ipp M, Stevens BB, Koren G. Effects of neonatal circumcision on pain response during vaccination in boys. *Lancet* 1995; **345**: 291–2
14 Anand KJS, Hickey PR. Pain and its effects in the human neonate and fetus. *N Engl J Med* 1987; **317**: 1321–29
15 Anders T, Chalemian R. The effects of circumcision on sleep-wake states in human neonates. *Psychosom Med* 1974; **36**: 174–9
16 Odent M. Learned helplessness: a concept of the future. In Denniston GC, Milos MF, eds, *Sexual Mutilations – A Human Tragedy.* Chapt. 10. New York: Plenum Press, 1997: 121–4
17 Kaplan GW. Complications of circumcision. *Urol Clin North Am* 1983; **10**: 543–9
18 Robson WL, Leung AK. The circumcision question. *Postgrad Med* 1992; **91**: 237–44
19 Audry G, Buis J, Vazquez MP, Gruner M. Amputation of penis after circumcision — penoplasty using expandable prosthesis. *Eur J Pediatr Surg* 1994; **4**: 44–5
20 Yilmaz AF, Sarikaya S, Yildiz S, Buyukalpelli R. Rare complication of circumcision: penile amputation and reattachment. *Eur Urol* 1993; **23**: 423–4
21 Bergeson PS, Hopkin RJ, Bailey RB Jr, McGill LC, Platt JP. The inconspicuous penis. *Pediatrics* 1993; **92**: 794–9
22 Craig JC, Grigor WG, Knight JF. Acute obstructive uropathy — a rare complication of circumcision. *Eur J Pediatr* 1994; **153**: 369–71
23 Cohen HA, Drucker MM, Vainer S *et al.* Post-circumcision urinary tract infection. *Clin Pediatr Phila* 1992; **31**: 322–4
24 Menahem S. Complications arising from ritual circumcision: pathogenesis and possible prevention. *Isr J Med Sci* 1981; **17**: 45–8
25 Mor A, Eshel G, Aladjem M, Mundel G. Tachycardia and heart failure after ritual circumcision. *Arch Dis Child* 1987; **62**: 80–1
26 Williams N, Kapila L. Complications of circumcision. *Br J Surg* 1993; **80**: 1231–6
27 Warren JP. NORM UK and the medical case against circumcision. In Denniston GC, Milos MF, eds, *Sexual Mutilations — A Human Tragedy.* Chapt. 7. New York: Plenum Press, 1997: 96–8
28 Pollack M. Redefining the sacred. In Denniston GC, Milos MF, eds, *Sexual Mutilations — A Human Tragedy.* Chapt. 16. New York: Plenum Press, 1997: 163–72
29 Pollack M. Circumcision: A Jewish Feminist Perspective. In Weiner K, Moon A, eds, *Jewish Women Speak Out.* Seattle: Canopy Press, 1995: 185
30 Ganzfried S. (translated Goldin HE). *Kitzur Shulchan Aruch, Code of Jewish Law.* Chapt 163, New York: Hebrew Publishing Co. 1961: 44
31 Encyclopedia Judaica. Vol. 13. Jerusalem: Keter Publishing House 1971: 510
32 Maimonides M. *The Guide for the Perplexed.* (Translated by Friedländer M). New York: Dover Publications, 1904, 1956: 378
33 Schmool M, Miller S. Women in the Jewish Community: Survey Report. London: Adler House, 1994: 23
34 Sacks J. BBC Radio 4, Interview 19th Sept. 1992

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